

Going It Alone: Oregon’s Healthier Oregon Medicaid Expansion Without Federal Funds

Abstract

Background: Oregon’s Healthier Oregon expansion expanded full-benefit Medicaid-like coverage to income-eligible residents regardless of immigration status. Evidence on these state-funded immigrant coverage expansions remains limited because most public surveys identify noncitizens rather than undocumented residents.

Methods: This analysis constructs a 2018-2024 ACS state-year panel for low-income noncitizen adults ages 19-64. The public-data analysis compares Oregon with a synthetic donor state constructed from states without comparable adult immigrant coverage policies. Outcomes are uninsurance, Medicaid or means-tested public coverage, any public coverage, and private coverage. The primary estimates use ridge-augmented synthetic control and are interpreted as an activation screen.

Results: In the post-policy period, uninsurance was 22.8 percent in Oregon, compared with an ASCM counterfactual of 42.5 percent, a gap of -19.7 percentage points. Medicaid or means-tested public coverage was 41.1 percent, compared with a counterfactual of 27.5 percent, a gap of +13.6 percentage points.

Conclusions: Oregon is the strongest public-data signal in this batch, with a large drop in uninsurance and a large increase in Medicaid coverage in 2024. The result is promising, but the private-coverage increase and short post-period make donor auditing and estimator comparison especially important.

Introduction

States have increasingly used state funds to close eligibility gaps left by federal Medicaid rules for noncitizens. Oregon’s Healthier Oregon expansion is a particularly useful policy case because it created a discrete eligibility change for a clearly defined low-income noncitizen target population. The policy question is not only whether state-funded expansions increase enrollment, but whether they measurably reduce uninsurance in population survey data that are available quickly enough for policy evaluation.

This paper asks whether Oregon’s Healthier Oregon expansion, effective July 1, 2023, changed coverage among low-income noncitizen adults ages 19-64. The analysis is deliberately framed around the public-data estimand: ACS noncitizens in the exposed age and income cell. That target does not perfectly identify undocumented residents, but it captures the population group most likely to be moved by the policy.

This study provides an early public-data evaluation of the coverage response to a state-funded immigrant coverage expansion. It does not replace the remaining design work: future work should audit the donor policy calendar, compare ASCM with SDID and generalized synthetic control, and add access or utilization outcomes if public survey data can support them.

Methods

Data and Population

The analysis uses the IPUMS ACS 1-year extract for 2018-2024. The sample retains household residents who are noncitizens, have family income from 1 to 200 percent of the federal poverty line, and fall in the exposed age band: ages 19-64. Person-level records are collapsed to state-year cells using ACS person weights.

The primary outcomes are uninsurance, Medicaid or means-tested public coverage, any public coverage, and private coverage. Insurance variables are harmonized from the staged IPUMS extract, which stores insurance indicators as binary 0/1 fields.

Policy Timing

The treatment is Oregon’s Healthier Oregon expansion, effective July 1, 2023. The primary post-treatment window is 2024, with 2023 treated as a transition year. The donor pool excludes states with comparable adult immigrant-coverage policies during the analysis window or other policy overlap documented in the source file.

Statistical Analysis

For each outcome, the analysis estimates a state-level synthetic-control model and a ridge-augmented synthetic-control model. The donor weights minimize pre-treatment differences between Oregon and a convex combination of donor states. The augmented model then uses ridge regression on donor pre-treatment histories to partially correct interpolation bias when Oregon is not exactly in the donor convex hull. The tables report treated observed values, ASCM counterfactuals, gaps in percentage points, pre-treatment RMSPE, and a placebo-in-space rank statistic for uninsurance.

Results

Table 1 describes the target-population coverage panel. Figure 1 shows raw pre-policy coverage trends for Oregon and the donor-pool mean. In the post-policy period, the ASCM estimates show lower uninsurance and higher Medicaid coverage in the treated state relative to its synthetic counterfactual.

For uninsurance, the observed post-period value was 22.8 percent, compared with an ASCM counterfactual of 42.5 percent, for a gap of -19.7 percentage points. For Medicaid or means-tested public coverage, the observed post-period value was 41.1 percent, compared with an ASCM counterfactual of 27.5 percent, for a gap of +13.6 percentage points. For any public coverage, the observed post-period value was 43.1 percent, compared with an ASCM counterfactual of 28.1 percent, for a gap of +15.0 percentage points. For private coverage, the observed post-period value was 39.5 percent, compared with an ASCM counterfactual of 31.1 percent, for a gap of +8.4 percentage points.

The direction of the Medicaid and uninsurance estimates is consistent with the expected first-stage effect of expanding full-benefit coverage. The private-coverage result is secondary and should be interpreted cautiously because survey coverage categories can overlap and because private coverage may respond to composition, labor-market, or marketplace changes not fully captured by the donor pool.

Discussion

Oregon is the strongest public-data signal in this batch, with a large drop in uninsurance and a large increase in Medicaid coverage in 2024. In practical terms, the results suggest that the ACS can detect coverage changes in the low-income noncitizen target cell soon after implementation, at least in this early synthetic-control screen.

The policy implications are straightforward but should remain measured. State-funded immigrant coverage expansions appear capable of moving public coverage and uninsurance in survey data, which matters for states considering expansions or retrenchments. However, the analysis does not identify undocumented residents directly and does not yet measure utilization, access, affordability, or safety-net financial outcomes.

Several limitations are important. First, ACS citizenship does not distinguish undocumented immigrants from lawful permanent residents, visa holders, refugees, or other noncitizen groups. Second, the post-period is short. Third, donor-pool contamination is possible because immigrant coverage policies are changing quickly across states. Fourth, pre-fit is very strong in several outcomes, which is encouraging for matching but also a reminder that many donor states and a short pre-period can produce fragile synthetic controls. Finally, the current analysis is an activation screen; publication-ready inference should add estimator comparisons and sensitivity checks before advancing causal language.

Future work should add SDID, generalized synthetic control, and matrix-completion estimates; audit state immigrant coverage policies year by year; test alternative age and income bands; and add access or utilization outcomes if public survey sources can support them.

Conclusion

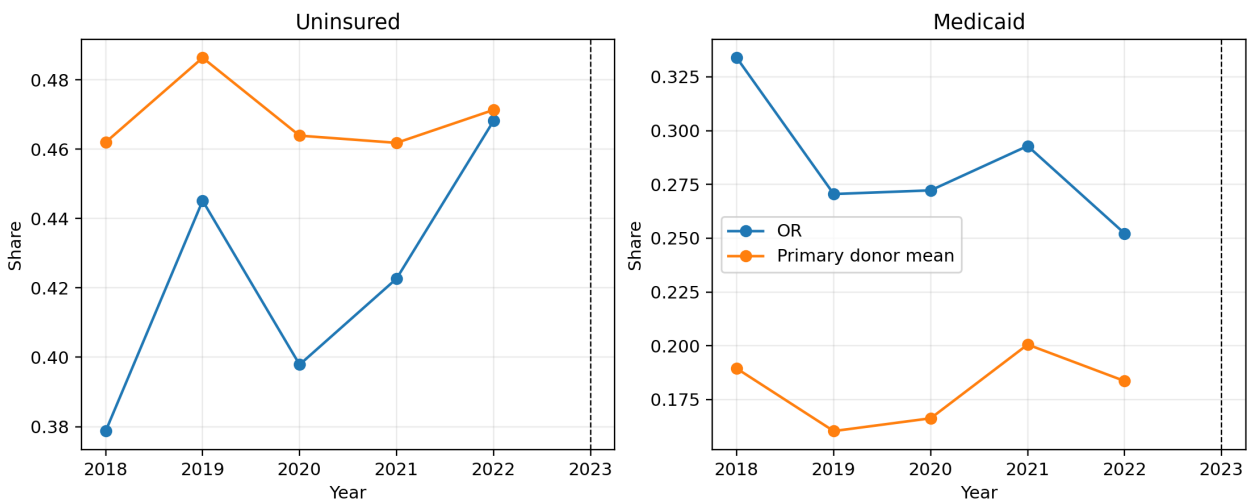
Oregon's Healthier Oregon expansion is associated in this early public-data screen with lower uninsurance and higher Medicaid coverage among low-income noncitizen adults ages 19-64. The result is promising enough to advance, but the manuscript should stay in a cautious working-paper frame until the donor policy audit and estimator-comparison checks are complete.

Sources

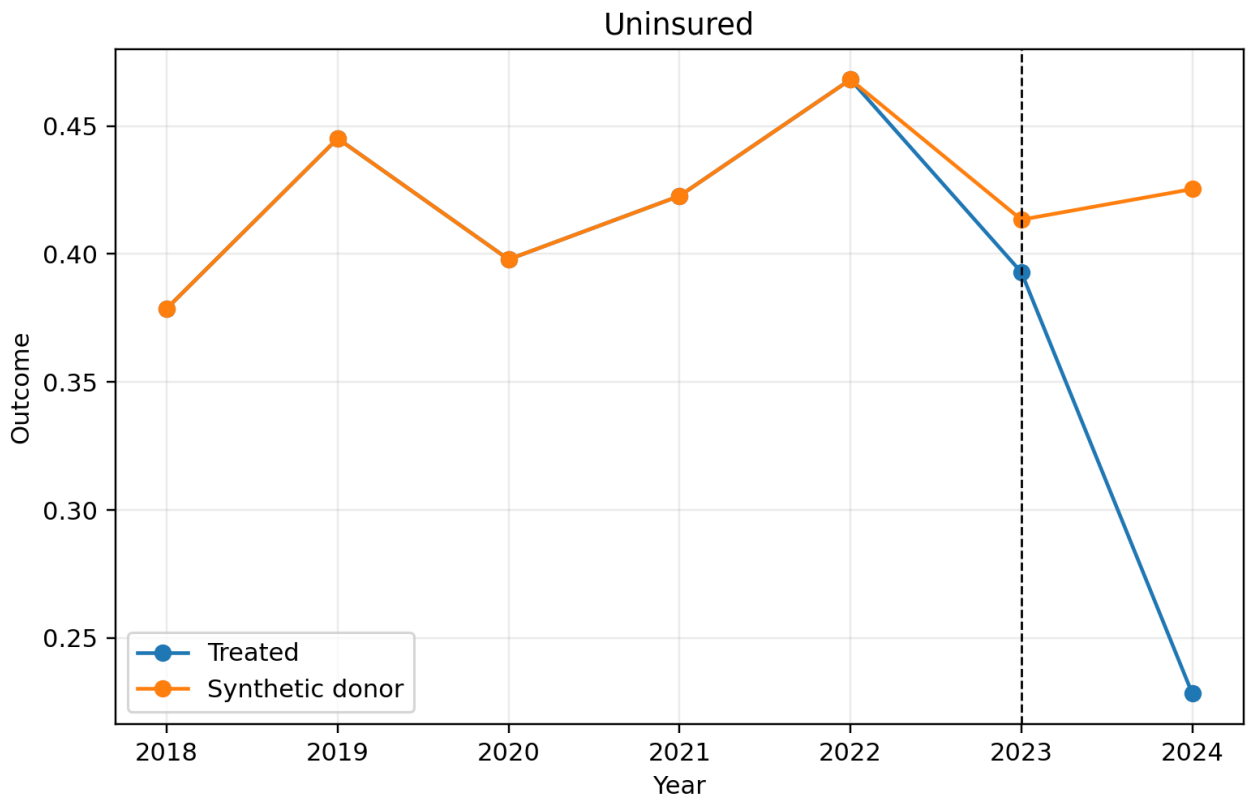
- Oregon Health Authority Healthier Oregon: <https://www.oregon.gov/oha/OHP/pages/healthier-oregon.aspx>
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Appendix

Appendix Figure A1. Raw ACS Pre-Trends



Appendix Figures A2-A5. Treated and Synthetic Paths



, the replication workflow, the replication workflow, and the replication workflow.

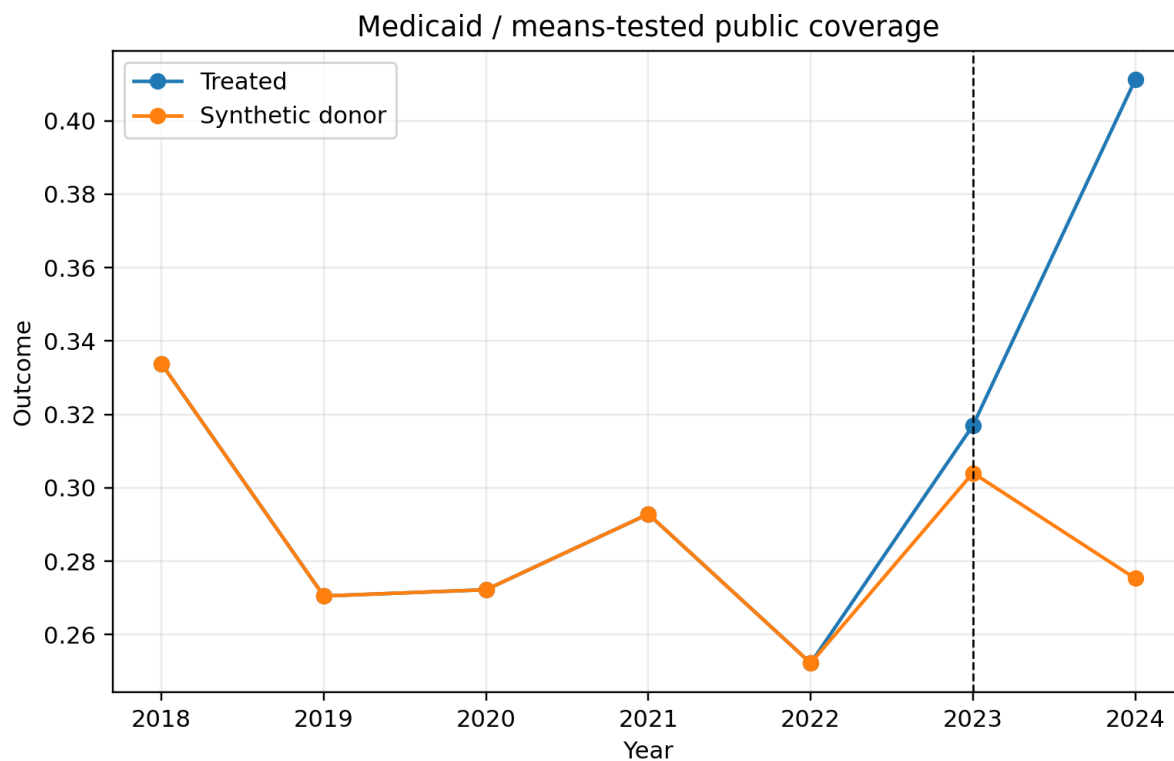


Figure 1: Synth Path Medicaid

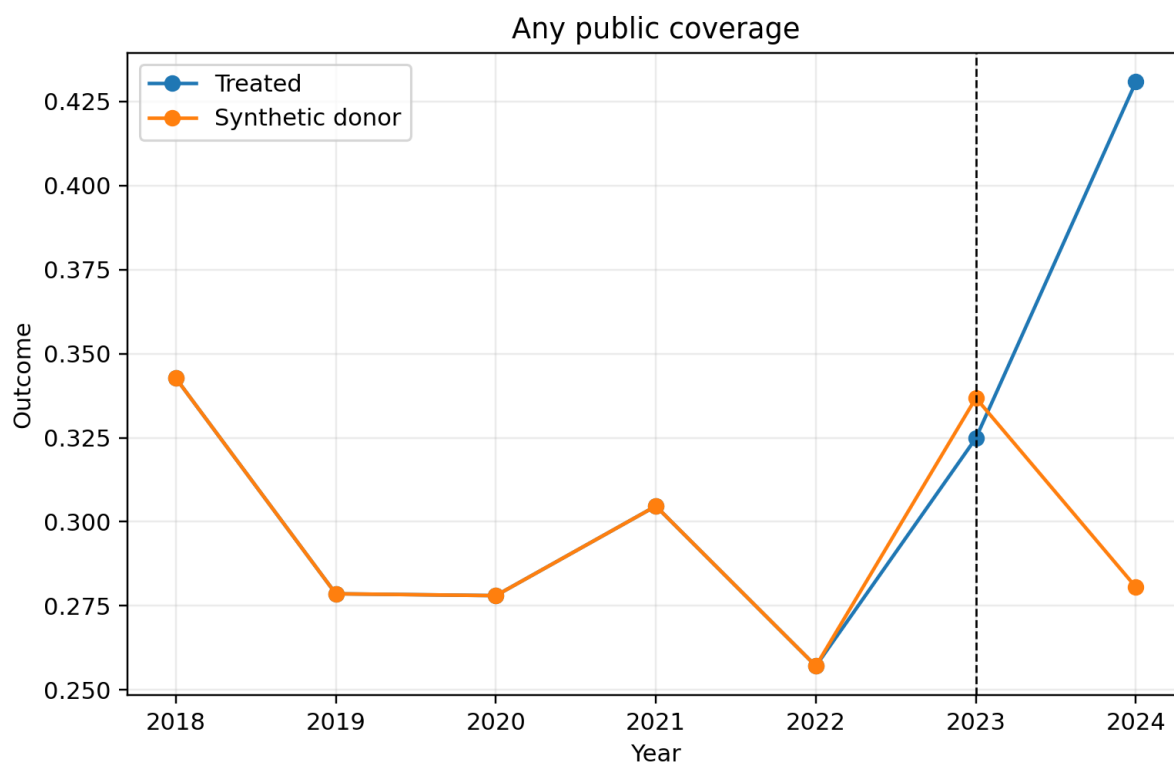


Figure 2: Synth Path Any Public

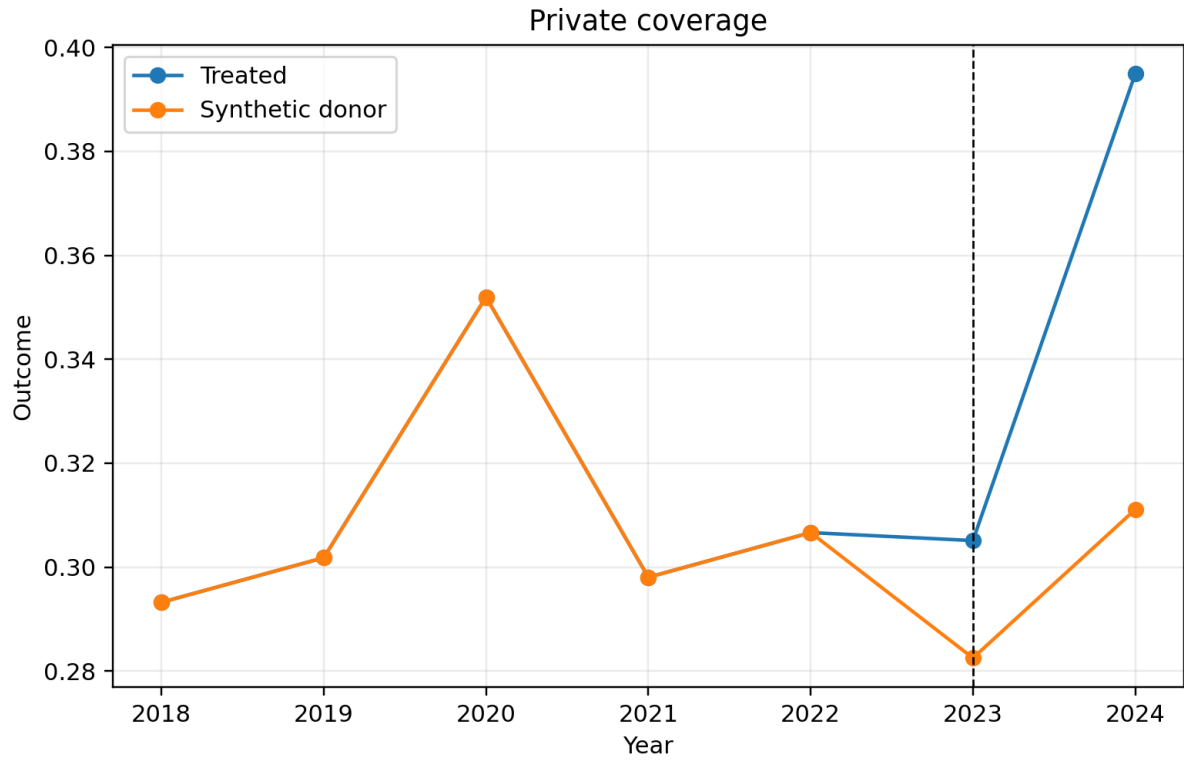
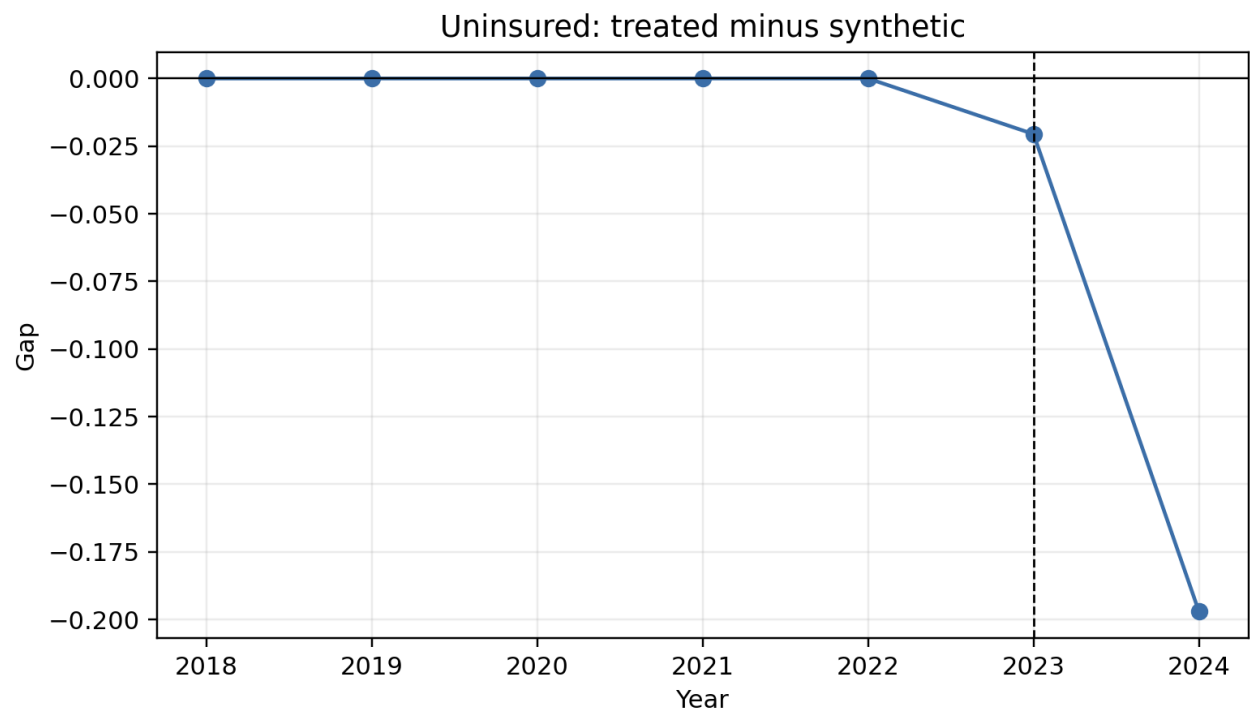


Figure 3: Synth Path Private Coverage

Appendix Figures A6-A9. Gap Paths



, the replication workflow, the replication workflow, and the replication workflow.

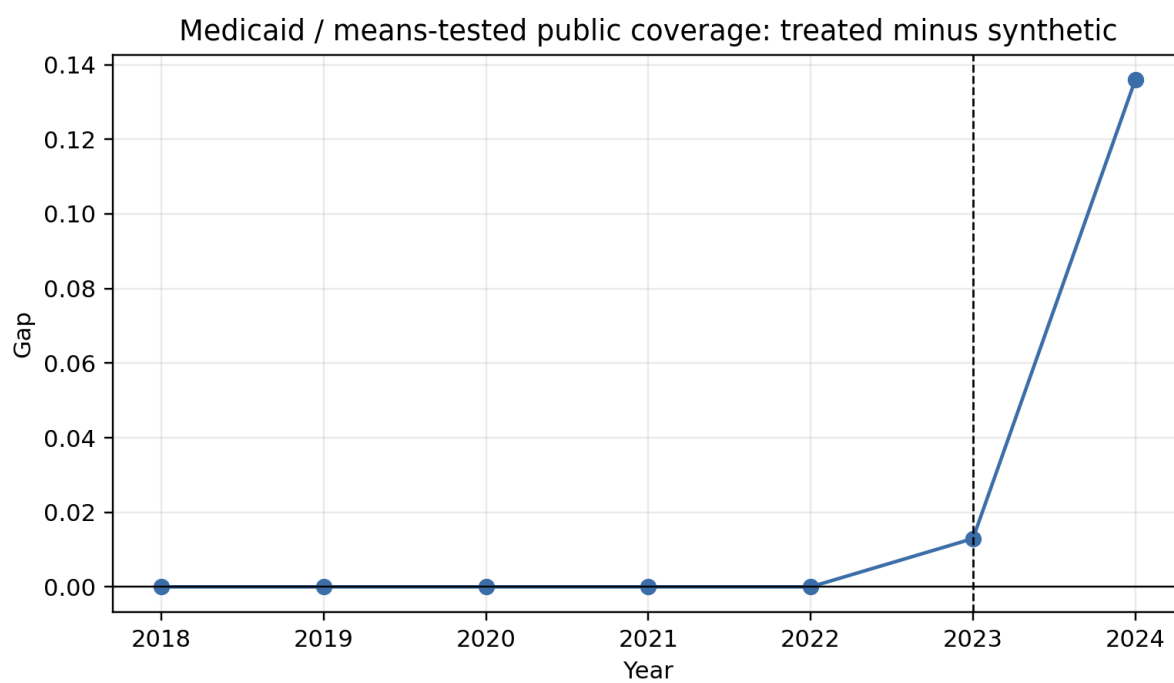


Figure 4: Gap Path Medicaid

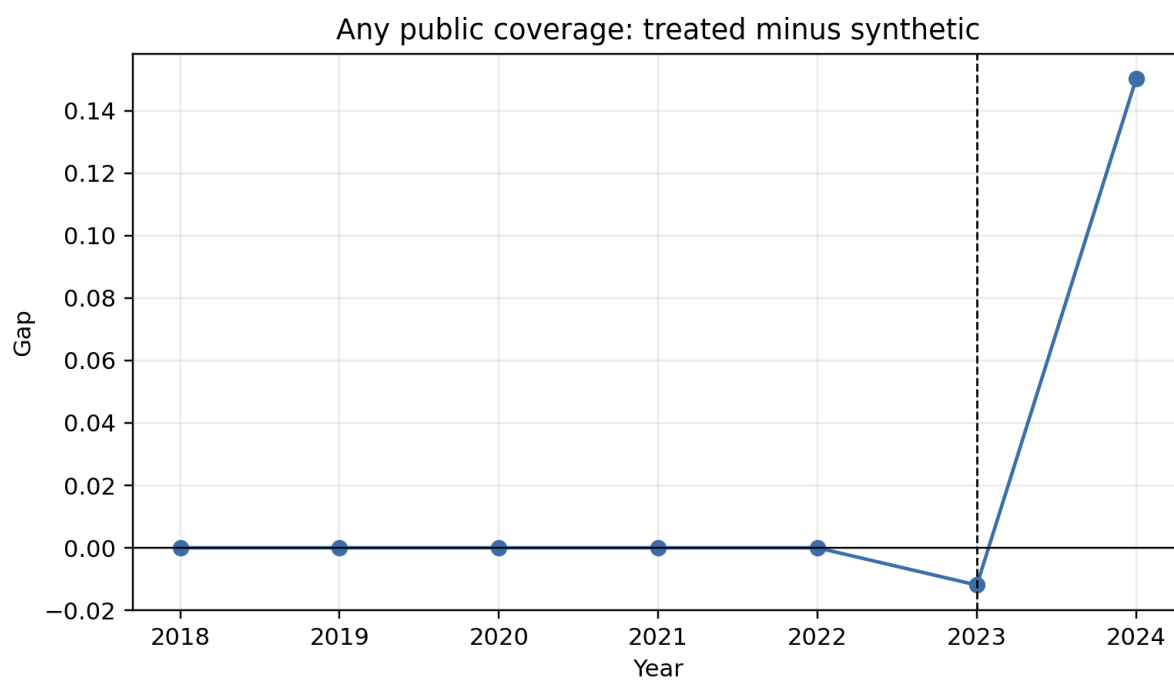


Figure 5: Gap Path Any Public

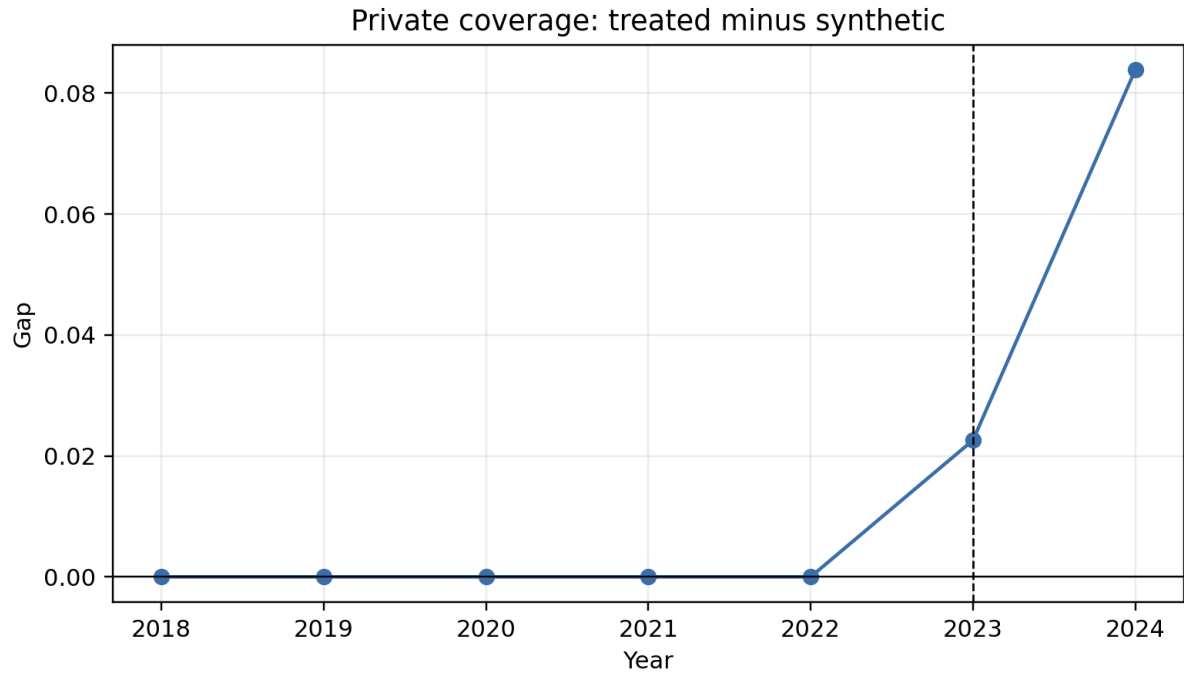


Figure 6: Gap Path Private Coverage

Appendix Table A1. Donor Weights

See the replication workflow and the replication workflow.

Appendix Table A2. Placebo-in-Space Diagnostics

See the replication workflow.